

Discharge Summary

Dear Colleagues,

Mr. Karl Hoffmann, DOB 06/28/1953, was admitted on 05/14/2024 for laparoscopic left hemicolectomy following colonoscopy-confirmed sigmoid adenocarcinoma. Preoperative CT staging showed a 3.8 cm sigmoid mass with two suspicious perisigmoid lymph nodes and no distant metastases.

Primary Diagnosis: Colorectal carcinoma (C20) — Tumor staging: T3 N2 M0

Selected laboratory results:

HbA1c	7.1%
CEA	18 ng/mL
Creatinine	1.0 mg/dL

Past Medical History: Mr. Hoffmann has a 12-year history of type 2 diabetes managed with sitagliptin 100 mg and metformin 500 mg twice daily; most recent HbA1c 7.1%. Arterial hypertension since 2009 on lisinopril 10 mg and hydrochlorothiazide 12.5 mg. Mild CKD stage 2, stable creatinine 1.0 mg/dL. Ex-smoker, 10 pack-years, cessation 2005. No alcohol. Right inguinal hernia repair 2014. No drug allergies.

Physical Examination on Admission: Height 176 cm, weight 91 kg, BMI 29.3 kg/m². BP 138/84 mmHg, HR 74 bpm, temperature 36.6°C. General condition satisfactory. Abdomen soft, mildly tender in the left lower quadrant on deep palpation. Digital rectal exam normal. No lymphadenopathy.

Operative Procedure: Laparoscopic sigmoid resection with primary anastomosis under general anaesthesia on day 1. Operative time 140 minutes; blood loss less than 100 mL. No intraoperative complications.

Postoperative Course: ERAS protocol applied. Clear fluids day 1, soft diet day 2. Drain removed day 3. Bowel function returned day 4. Wound inspection daily —

clean throughout. VTE prophylaxis continued.

Histopathology: Moderately differentiated adenocarcinoma pT3 pN2 (4 of 16 nodes involved) M0. Resection margins clear (R0). MSI-stable. KRAS exon 2 codon 12 mutation detected. BRAF wild-type. Full report appended.

Nursing and Stoma Care: Although no stoma was formed, the stoma nurse specialist reviewed the patient preoperatively as a precaution given the anastomosis level and performed siting assessment. Postoperative port sites inspected daily, remained clean without infection or dehiscence. Written wound care instructions and community nursing contact provided.

Nutritional Assessment and Recovery: Dietitian review on day 3 postoperatively. High-calorie, high-protein diet recommended to support wound healing. Good appetite by day 4. Patient tolerating full soft diet at discharge. Advised to report any significant change in bowel habit promptly.

Medication at Discharge: Lisinopril 10 mg, hydrochlorothiazide 12.5 mg, metformin 500 mg twice daily (sitagliptin held pending oncology review), enoxaparin 40 mg subcutaneously once daily for 4 weeks, pantoprazole 20 mg.

Follow-up: MDT discussed case — adjuvant mFOLFOX6 chemotherapy recommended given pN2 disease and KRAS mutation. Oncology referral sent. GP in 1 week for wound review. Colorectal surgery in 6 weeks.

Perioperative Assessment and Risk Stratification:

Prior to surgery, Mr. Hoffmann underwent a comprehensive preoperative assessment. Cardiac risk was stratified using the Revised Cardiac Risk Index — score 2 (intermediate risk) given diabetes and the planned intraperitoneal surgery. A preoperative ECG showed normal sinus rhythm with no ischaemic changes. Renal function was reviewed — creatinine 1.0 mg/dL, eGFR 68 mL/min — deemed acceptable. Anaesthetic review was completed and the patient assessed as ASA grade II.

Multidisciplinary Tumour Board Discussion:

The case was presented at the colorectal multidisciplinary tumour board meeting on 05/21/2024. Members included the colorectal surgeon, oncologist, radiologist,

pathologist, and specialist nurse. The board reviewed the staging CT and the final histopathological report confirming pT3 pN2 M0, MSI-stable, KRAS exon 2 codon 12 mutation detected, BRAF wild-type. The recommendation was for adjuvant chemotherapy with modified FOLFOX6 given node-positive disease and KRAS mutation status. Anti-EGFR therapy is not indicated in KRAS-mutated tumours. The patient and his wife were fully informed of the MDT recommendation and expressed a wish to proceed with adjuvant chemotherapy. A written summary of the MDT discussion was placed in the medical record and provided to the referring physician.

Patient and Carer Information Session:

Mr. Hoffmann and his wife received a comprehensive information session delivered by the colorectal clinical nurse specialist on day 6. Topics covered: the histopathology results and their implications, the rationale and schedule for adjuvant chemotherapy, the expected side effect profile of FOLFOX6, colonoscopic surveillance scheduling (at 1 year, 3 years, and 5 years), dietary advice for the recovery period, and the contact details for the colorectal specialist nursing team and the chemotherapy day unit. Written information on colorectal cancer and chemotherapy was provided.

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